

1. Introduction

As over 100 000 sites offer health related information, trying to get health information from the internet is like drinking from a fire hose, you don't even know what the source of the water is (McLellen, 1998). False and low-quality health information can harm users' health and even jeopardize their lives.

The development of public reporting has been a result of a change in the concept of health from disease treatment to disease prevention and healthy lifestyle cultivation, as well as the increased demand for health services and the growing number of individuals worldwide who suffer from functional or cognitive impairment since the pneumonia epidemic. Subsequently, the public has paid more attention to health. Public reporting offers patients an official method of obtaining health information, thereby improving and quickening the process of obtaining medical information, as a result of the rapid advancement of the Internet. The Internet has the potential to significantly influence the level of information that healthcare consumers possess, thereby enabling them to make more informed decisions when selecting nursing facilities (Wald et al., 2007). This is in contrast to the traditional approach of merely processing information. Public reporting has been thoroughly researched and is highly beneficial as an inquiries tool. Numerous studies concentrate on the enhancement of patient-centred healthcare in hospitals, which involves the "respecting and responding to individual patient preferences, needs, and values," the guarantee that patient values inform all healthcare decisions, and the provision of patients with the necessary education and support to actively engage in their own care. (Institute of Medicine, 2001)

Concurrently, the emergence and development of public reporting as a new health service model are being driven by social factors, including the aging of the population, the increase in the number of patients with chronic diseases, and the unequal distribution and allocation of medical and healthcare resources (Jie et al., 2016). For instance, research has demonstrated that individuals with deteriorating health are more

inclined to seek information to assist in managing their care and are more proactive in their search for information. Consequently, they are more inclined to identify that physician quality information in public reporting, women and individuals with higher education levels are more likely to be aware of this, and individuals with chronic diseases are more proactive in obtaining and understanding medical information (Abraham et al., 2006). Individuals who are married or cohabitating are considerably more inclined to conduct online searches for health information. (Suziedelyte A, 2012; Renahy, 2005-2006)

On the one hand, users' health and even their lives can be negatively impacted by misleading and low-quality health information. On the other hand, health information, as it applies to health maintenance, can not only offer patients a deeper understanding of their physical condition, but also offer healthy people guidance for daily maintenance.

Nevertheless, these governance measures are predominantly implemented from the perspective of the public reporting website, which is a form of external governance. In contrast to the governance measures implemented by platforms, individuals, as the subject of governance, focus on achieving self-governance of online health information through their autonomy and independence, making them an important subject of information governance. They play multiple roles in the chain of production, dissemination, and utilisation of health information. For instance, Wang et al. (2018) illustrated that users' autonomous behaviours, including questioning and seeking confirmation, could potentially contribute to greater scepticisms of misleading information among other users, facilitate the early emergence of the truth, and prevent additional users from being misled (Weil et al., 2002).

In the comparable way that TripAdvisor and Amazon share consumer products and travel experiences, it is a wise decision to offer patients the opportunity to share anecdotal reviews or reports (stories told in their own words) about their experiences

with care. It comprehends that the public frequently employs anecdotal narrative reports to make product purchases, and that consumer evaluations have an impact on these decisions (Anderson, 2012). There is an increasing interest in evaluating the feasibility of integrating narrative evaluations into public reporting of health care provider performance (Lagu & Lindenauer, 2010; Schlesinger, Kanouse, Martino, Shaller, & Rybowski, 2013). Berwick et al. (2003) elucidated that citizens identify outcomes that are pertinent to them, learn about the various performance levels of surgeons, and select their surgeon when they make decisions to enhance their health care. The citizens are wholly responsible for selecting high-quality health care and social service care.

The cognizance and recognition of patients' personal preferences, requirements, and values are the defining characteristics of patient-centered healthcare. The report from the Institute of Medicine ensures that patient values are the guiding principles for all healthcare decisions and provides patients with the requisite education and support to actively participate in their own care (Rockville, 2011). Research has shown that individuals who are experiencing a decline in their health will be more motivated to acquire information to help them manage their own care and will be more active in their pursuit of information. The research on public reporting is relatively mature, and some scholars have conducted research on the use of information on healthcare provider performance to encourage hospitals to enhance quality (Wilder, 2021; Prang, 2021; Poldrugovac, 2022). Furthermore, certain academics have examined the accuracy and validity of health data in the public sector for the purpose of evaluating the performance of an organization or health care provider (Emmert, 2017). Nevertheless, the majority of research continues to concentrate on the enhancement of the quality of treatment in hospitals, health data, and reputation. The user level was only addressed in two peer-reviewed studies. Research is still insufficient, Kolb et al., (2015) conducted the study on doctor evaluation websites offer patients a novel method of providing feedback. Schlesinger (2015) conducted the study on consumer's narrative in conjunction with quantified metrics.

The research subjects of the reviewed articles were mapped, and three primary stakeholder groups were identified: health care experts and providers, patients with illnesses, and consumers. The primary purpose of public reporting is to provide users with a fundamental understanding of health information and healthcare. The primary applications of public reporting for patients with ailments are to gain knowledge about the disease, its complications, and their daily diet and lifestyle. Learning and comprehending the disease can empower patients to identify changes in their bodies and initiate modifications to their diet and lifestyle as a result of individual differences and variations in diet and lifestyle. The primary purpose of public reporting for health care professionals and providers is to enhance quality. By providing financial incentives, the use of public reporting websites can assist them in providing public reporting services more effectively. Simultaneously, enhanced public reporting may also enhance market share and reputation. This study contributes to the development of future research endeavours by mapping the research themes, analysing the focal constructs of previous studies, synthesizing future research orientations, and ultimately reflecting on the findings and proposing new research areas (Kitchenham, 2010; Petersen, 2015)

The subsequent sections of the paper are organized as follows. The subsequent section addresses the background and related works, followed by the research methodology and the specifics of the article selection process. The result of the analysis is then reported. Lastly, the implications, limitations, and potential directions for future research are addressed. The quality assessment criteria utilized to determine the articles to be included in the review are detailed in Multimedia Appendix 1.

2. Research background

Public reporting is the process of ensuring that healthcare quality information is publicly available and transparent to consumers (Kumpunen, 2014). By providing data as feedback to organizations or health care workers who are evaluating their

performance, it is a method for the public to obtain information about the quality of health care services. This information is used to motivate improvements and protect patients by differentiating the performance of hospitals, doctors, or other health care providers (McKibben et al., 2006). The objective of the strategy is to enhance transparency and informed provider selection, encourage quality enhancement, and ensure that providers are held accountable for the care they deliver (Reinhard, 2019).

Depending on the indicators that are measured and reported through various channels, public reporting is anticipated to facilitate enhancements in safety, effectiveness, and/or responsiveness. This information is typically accessible via the Internet, but it is also available in paper and other formats. In contrast to scattered health information, publicly reported quality-related information is non-anonymous, identifiable professionals and providers (e.g., individuals, institutions), and it is systematically collected comparative data to enable information and inquirers to query and compare different health information data. Quality-related information is publicly reported to the public, as is health information (Reinhard et al., 2019). Typically, there are transparent methods for enhancing publicly reported information. Consumers evaluate PPR data and select purveyors of superior quality over those of inferior quality during the selection process. Organizations enhance their performance by identifying areas of weak performance during the change path. Suppliers' incentives to preserve or expand their market share are contingent upon these pathways (Berwick et al., 2003). PPR can have a detrimental effect on the public image of a provider or organization in the reputation path. Consequently, reputational concerns will cause a provider or organization to engage in quality enhancement activities in order to safeguard or enhance its public image (Hibbard, 2005).

On a global scale, an increasing number of individuals are dependent on others to manage their daily activities due to functional or cognitive impairments. When they are compelled to relocate to a long-term care facility. They subsequently require access to hospital information, which can be a highly effective method for assisting patients in

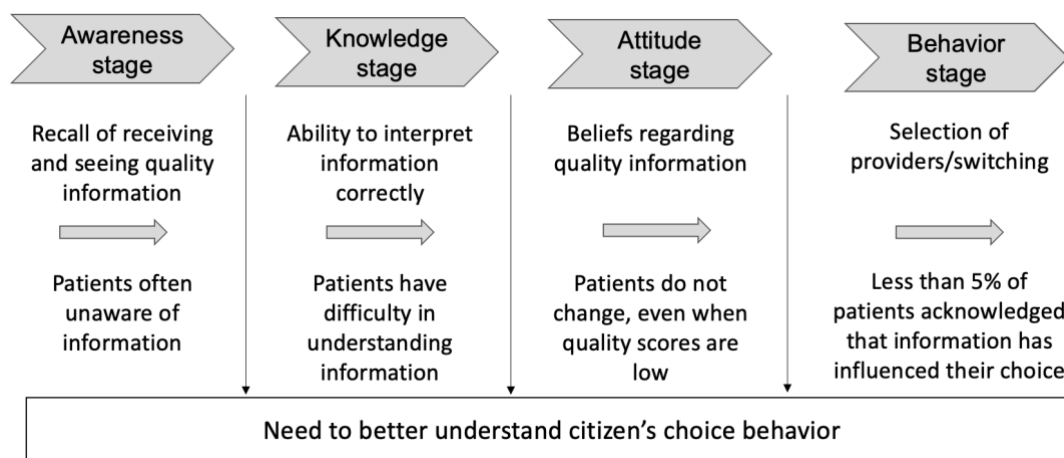
comprehending the processes and outcomes of the hospital. Soroush et al. (2018) The direct generation of ratings and evaluations by patients and users is a relatively new phenomenon in quality reporting. Patient evaluations have been integrated into provider profiles on the government-operated website NHS Choices in the United Kingdom. The website provides patients with the ability to provide narrative feedback on a variety of care providers, such as general practitioners, hospitals, and long-term care providers. Additionally, it offers feedback and connections to supplementary websites. Additionally, the Dutch Care Map (Zorgkaart Nederland; www.zorgkaartnederland.nl) offers hyperlinks to a substantial commercial website that offers patient and user evaluations (Stephanie, 2014). The question of whether and to what extent the information users receive from these sources is actually beneficial to them, given the overwhelming amount of information available, and whether it has an impact on user behavior is raised by the fact that plenty of people are procuring information about the quality of health care from social media and commercial source. Anecdotal evidence from other social media platforms, such as Twitter, indicates that hospitals with a reputation for providing high-quality care are the most effective at engaging with patients on social media. Of course, the solution for hospitals that provide substandard care is not to establish a Facebook page or Twitter account; rather, utilizing these platforms is indicative of a general propensity to interact with patients, incorporate feedback, and implement improvements as a result.

In the same way that TripAdvisor and Amazon share consumer products and travel experiences, it is a wise decision to offer patients the opportunity to share anecdotal reviews or reports (stories told in their own words) about their experiences with care. We are aware that the public frequently employs anecdotal narrative reports to make product purchases, and that consumer evaluations can significantly impact these decisions (Anderson, 2012). For instance, Wang BR et al. (2018) observe that users' autonomous behaviors, such as inquiry and confirmation seeking, may encourage other users to be more skeptical of misleading information, promote the early emergence of the truth, and prevent other users from being misled (Weil and Kennedy, 2002). There

is an increasing interest in evaluating the feasibility of integrating narrative evaluations into public reporting of healthcare provider performance (Lagu & Lindenauer, 2010; Schlesinger, 2013). Since 2007, the NHS Choices website has been the repository of patient evaluations and reports from the UK NHS, offering a glimpse into the extensive healthcare industry applications. Patients who have experienced notably positive or negative experiences may be more inclined to provide feedback in this manner. Berwick et al (2003) stated that citizens identify outcomes that are pertinent to them, comprehend the various performance levels of surgeons, and select their surgeons when making decisions to enhance their health care. Citizens are wholly responsible for selecting high-quality health care and social service care. Despite the actions of policy makers in public disclosure of monitoring, evaluation, and policy assessment, these issues have also been identified in research. De et al. (2009) have characterized misinterpretation of information, manipulation of data, and an insufficient focus on the subject matter being measured. Nevertheless, the impact of misunderstanding health information on health and life is irreversible, in contrast to other types of information. Therefore, what precisely transpires when patients review the information during the medical decision-making process?

Berwick et al. (2003) elucidated that citizens identify the outcomes that are pertinent to them, comprehend the various performance levels of surgeons, and subsequently select their surgeons when making decisions to enhance their health care. Nevertheless, Faber et al. (2009) demonstrated that the health information literacy of each individual is unique. Figure 1 illustrates the various stages that consumers undergo when selecting a healthcare provider based on medical quality information, as illustrated by this model. A series of positive actions and reactions are initiated by an effective public transparency system, as per TAC. Initially, information disclosers furnish the public with information. Secondly, information consumers alter their behaviors and perceptions in response to new information. Third, in order to enhance their competitive edge, information disclosers modify their behaviors and perceptions in accordance with the actions of users (Reinhard et al., 2019).

Figure 1. consumers undergo when selecting a healthcare provider based on medical quality information.



For instance, the knowledge, attitude, and practice (KAP) model are a psychological cognitive model that defines knowledge as the capacity to acquire and apply information, while attitude is the outcome of a specific response to a specific situation. The two are causally related, with knowledge (information) serving as the foundation and attitude as the motivator. The fundamental perspective is that the accumulation of knowledge (information) will result in a certain degree of attitude changes (Chien-Yun et al., 2011). Consequently, we are of the opinion that attitudes are substantially influenced by the accessibility of information. What is the impact of public reporting of prescription quality indicators on prescription practices?

Typically, users must rely on their own knowledge to evaluate the facts of the disease when obtaining health information; however, their current knowledge is often restricted. Patients chose for clinicians who scored lower on other quality metrics when narratives were incorporated into their report cards (Schlesinger, 2012; Schlesinger, 2014).

Patients and users may make irrational decisions when utilizing social processes to make decisions, as they may take into account a variety of factors, such as their own past experiences and the opinions of others. For instance, an individual may not select

a hospital with a high rating solely because his grandmother passed away there (Marshall & McLoughlin, 2010). A significant proportion of the working-age adult population in numerous countries lacks the fundamental skills necessary to utilize information and communication technologies (ICTs), including the Internet and computers, to accomplish a variety of daily tasks (Stephanie et al., 2014). In addition to the significant number of middle-aged and geriatric individuals who are afflicted with chronic diseases, such as diabetes, they are also susceptible to complications from other diseases. The number of chronic diseases among Medicare beneficiaries increases with age, and approximately two-thirds of them have two or more chronic diseases. Beneficiaries who have a greater number of chronic diseases consult their physicians on an annual basis (Centers for Medicare and Medicaid Services, 2012). The value of public reporting information is derived from the difficulty of assessing these facts using personal knowledge.

It is still unclear how public reporting information influences user behavior, which information is beneficial to users, which information alters users' behavior, and whether the change in user behavior contributes to the improvement of public reporting information. There are numerous studies that investigate the enhancement of public reporting; however, there are few studies that have attempted to comprehend the influence of public reporting information on user behavior and the modifications in user behavior that occur as a result of receiving this information.

3. Methods

The systematic literature review aimed to determine the current scenario of public reporting research and the different parties involved in the healthcare industry. To that goal, we investigated the six particular research questions (RQs) indicated in Table 1.

Table 1. Research questions.

Number	Research question
RQ1 ^a	What is the present status of the research on public reporting in the health care area?
RQ2	What are the most important publication channels?
RQ3	What research methods have been used?
RQ4	What theoretical backgrounds and variables have been employed?
RQ5	What recurring research themes can be identified from the literature?
RQ6	What future research directions can be synthesized from the literature?

^aRQ: research question.

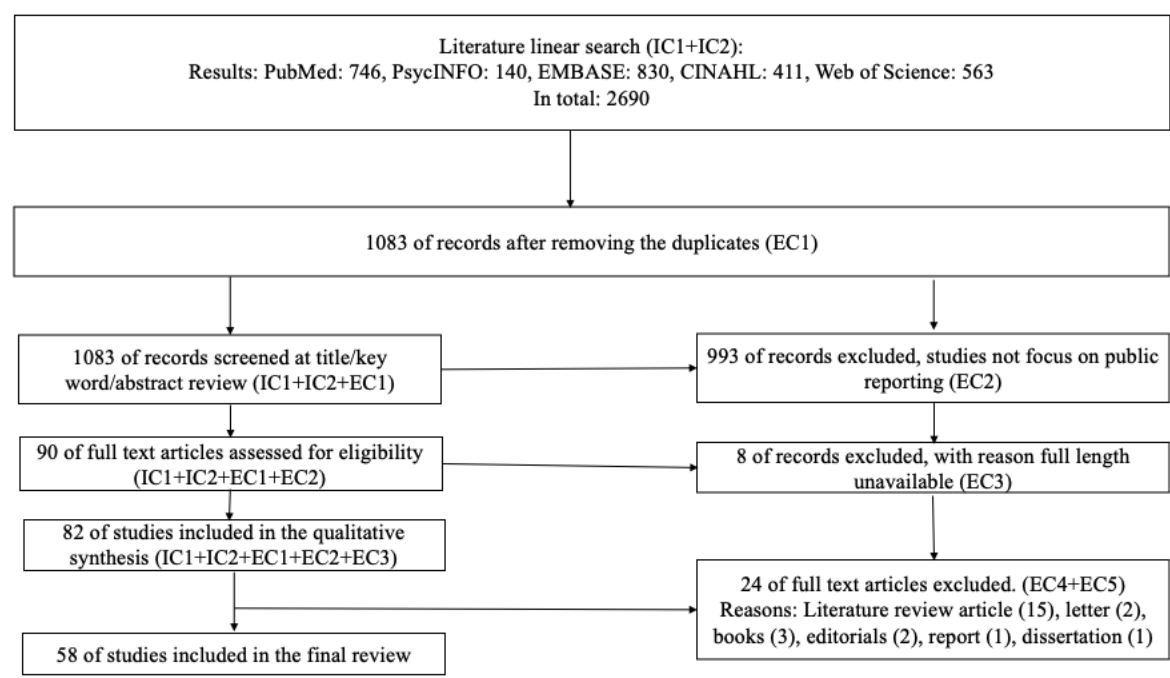
Research Design

A research literature review is "a systematic, explicit, and reproducible method for identifying, evaluating, and synthesizing the existing body of completed and recorded work produced by researchers, scholars, and practitioners" (Fink, 2013), which typically aims to identify new future research areas, facilitating theory development, closing saturated research areas, and categorizing and classifying studies in specific research streams (Okoli, 2015; vom Brocke et al., 2015; Webster & Watson, 2002). Since public reporting is crucial to hospital performance, a 2016 Health Affairs study found that no hospital received a high rating from any of the four major national rating systems: Consumer Reports, Leapfrog, Healthgrades, and U.S. News & World Report (Austin et al., 2015). Hospitals should be aware of the current state of public quality reporting, as well as hospital and physician ratings and data sources, and reporting transparency. This paper conducts a systematic literature review of public reporting in the healthcare field. This type of literature review summarizes previous knowledge and identifies gaps in a specific area of research (Paré et al., 2015). This paper closely follows the guidelines suggested by Webster and Watson (2002) and Okoli (2015) to conduct a systematic literature review and ensure that the results are clear, comprehensive, and replicable.

Based on the current work's topic of public reporting in the health care area. Initially, articles were retrieved from five academic databases: PubMed, PsycInfo, EMBASE, CINAHL, and Web of Science (WoS). These databases includes the fields of health, biomedicine, psychology, and medicine. The purpose of this search was to identify

eligible studies as broadly as possible. These databases was accessed via the school library to check that the user had logged in, preventing missing articles owing to users failing to log in to the school account. The number of search phrases for each database was recorded to verify that the logarithm of each subsequent step was correct. To search for articles as broadly as possible and prevent missing items during the search process, the number of search results, downloads for each database, and imports into the Endnote software were all tracked. The language restriction was set to English during the search to allow the general public to query and refer to it at any moment. Figure 1 depicts the phases in article retrieval as well as the amounts of articles.

Figure 1. Literature search steps. IC: inclusion criterion; EC: exclusion criterion.



Second, the following search terms were applied to search titles and abstracts of articles and to find subject-specific articles for the literature review: (“healthcare,” “health care,”) and (“public reporting,”). The current study’s authors searched 2690 publications in total, of which 746 in Pubmed, 563 in WoS, 411 in CINAHL, 830 in

Embase and 140 in PsycInfo. By importing into endnote software to delete duplicates, then manually remove duplicates and read the title to double-check for omissions.

Third, articles' abstracts, keywords, and titles were screened across the five databases based on the above keywords. The further screening of the 1083 articles for inclusion and exclusion was based on the inclusion and exclusion criteria in a two-step process. In the fourth step of applying practical screening criteria, the inclusion criteria and exclusion criteria has been shown in table 2. The literature review first screened the title and abstract of these 1083 articles to select articles to decide whether they concerned research on public reporting and then looked through the title/key words/abstract selected for forward and backward checking. After reading the abstracts and titles, 993 records were excluded for not matching the inclusion criteria (studies with the main topic (title/abstract/keywords) containing public report, studies available in English). In screening the titles of the 1,083 retrieved articles, the screening step includes whether the titles of these 1,083 articles contain the keyword "public report". If the title is unclear, read the abstract to discover if the research focusses on public reporting. If it just mentions public reporting, for example, if the article gives the recommendation to pay attention to public reporting or the article uses public reporting data to investigate other study, it should also be excluded.

Table 2. Inclusion and exclusion criteria

Inclusion criteria (IC)

IC1: Studies published in English

IC2: Studies with the main topic (title/abstract) containing public report

IC3: Full-texts of studies

IC4: Studies published in journals or scientific conference

Exclusion criteria (EC)

EC1: Eliminate duplicate studies from different searches

EC2: Studies whose main topic is not public reporting (Studies not focus on public reporting means that even if the abstract mentions public reporting, the article is not about public reporting).

EC3: Full text unavailable (studies were incomplete).

EC4: Studies that are not empirical studies (books, editorials, report, letters)

EC5: Literature review article

By further not fulfilling the inclusion criteria at the analysis level of full text, 8 articles were excluded, leaving 82 studies.

Forth, the types of articles were screened, the remaining 82 articles were screened in full text to determine whether they met inclusion criteria 4 (studies published in journals and conference proceedings), excluding 15 literature reviews and 9 conceptual papers (letter 2, books 3, editorials 2, report 1, dissertation 1) that did not contain any empirical analysis. Reviews and letters were excluded because they were not reporting original data.

In addition, 58 articles were read and analyzed in full. Their research methods and theories, research equipment, and research contexts were identified to establish a general understanding of these studies.

The search string had wide coverage to avoid missing any research of interest. In our search, there were no restrictions on publication time or study design. Studies published in languages other than English were excluded. The search strategy for each database is shown in Table 2, and the detailed explanation of these search strategies is provided.

Table 2. Databases and search terms

Database	Search terms
PubMed (746)	("healthcare"[Title/Abstract] OR "health care"[Title/Abstract]) AND "public report*" [Title/Abstract]
PsycInfo (140)	(TI healthcare OR AB healthcare OR TI "health care" OR AB "health care") AND (TI "public report*" OR AB "public report*")
EMBASE (830)	(healthcare OR "health care"):ab,ti AND ("public report*"):ab,ti
CINAHL (411)	(TI healthcare OR AB healthcare OR TI "health care" OR AB "health care") AND (TI "public report*" OR AB "public report*")
Web of science (563)	(TI=(healthcare OR "health care") OR AB=(healthcare OR "health care")) AND (TI="public report*" OR AB="public report*")

Finally, 58 articles were selected as the final sample.

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